

C. NO. 305023071705Q

Labcorp Diagnostics Pvt. Ltd.

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Labcorp Diagnostics Pvt. Ltd. Laboratory Test Report

Patient Name: **Mr. ZAHIQ ABBAS / TUJER**

Age/Gender : 23 Y/Male

Refer Client : LDPLJK2347 ZAINGAIR BLOOD COLLECTION C

Referred By : N/A

Doctor Name:

SampleType : Serum - JK3536287, Sod.Fluoride - F - JK3536285, Urine - , Whole Blood EDTA - JK3536286

PateintID : **LDPL/01076921**

Visit No : SRI2606220995

Collected On : 22/Jun/2026 05:08 PM

Received On : 22/Jun/2026 07:20 PM

Reported Date 22/Jun/2026 08:47 PM



HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
HBA1C -GLYCOSYLATED HEMOGLOBIN			
Hb A1C, GLYCOSYLATED Hb,wb edta by HPLC	5.14	%	Normal <5.7% Prediabetes 5.7% to 6.4% Diabetes 6.5% or higher
Estimated Average Glucose	100.82	mg/dL	68-125

INTERPRETATION

AS PER AMERICAN DIABETES ASSOCIATION (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	< 5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5

CLINICAL NOTES

In vitro quantitative determination of HbA1c in whole blood is utilized in long term monitoring of glycemia. The HbA1c level correlates with the mean glucose concentration prevailing in the course of the patient's recent history (approx - 6-8 weeks) and therefore provides much more reliable information for glycemia monitoring than do determinations of blood glucose or urinary glucose. It is recommended that the determination of HbA1c be performed at intervals of 4-6 weeks during Diabetes Mellitus therapy. Results of HbA1c should be assessed in conjunction with the patient's medical history, clinical examinations and other findings.

Some of the factors that influence HbA1c and its measurement [Adapted from Gallagher et al]

1. Erythropoiesis

- Increased HbA1c: iron, vitamin B12 deficiency, decreased erythropoiesis.
- Decreased HbA1c: administration of erythropoietin, iron, vitamin B12, reticulocytosis, chronic liver disease.

2. Altered Haemoglobin-Genetic or chemical alterations in hemoglobin: hemoglobinopathies, HbF, methemoglobin, may increase or decrease HbA1c.

3. Glycation

- Increased HbA1c: alcoholism, chronic renal failure, decreased intraerythrocytic pH.
- Decreased HbA1c: certain hemoglobinopathies, increased intra-erythrocyte pH.

4. Erythrocyte destruction

- Increased HbA1c: increased erythrocyte life span: Splenectomy.
- Decreased A1c: decreased RBC life span: hemoglobinopathies, splenomegaly, rheumatoid arthritis or drugs such as antiretrovirals, ribavirin & dapsone.

5. Others

- Increased HbA1c: hyperbilirubinemia, carbamylated hemoglobin, alcoholism, large doses of aspirin, chronic opiate use, chronic renal failure
- Decreased HbA1c: hypertriglyceridemia, reticulocytosis, chronic liver disease, aspirin, vitamin C and E, splenomegaly, rheumatoid arthritis or drugs

Note:

1. Shortened RBC life span -HbA1c test will not be accurate when a person has a condition that affects the average lifespan of red blood cells (RBCs), such as hemolytic anemia or blood loss. When the lifespan of RBCs in circulation is shortened, the A1c result is falsely low and is an unreliable measurement of a person's average glucose over time.

2. Abnormal forms of hemoglobin - The presence of some hemoglobin variants, such as hemoglobin S in sickle cell anemia, may affect certain methods for measuring A1c. In these cases, fructosamine can be used to monitor glucose control.

estimated Average Glucose (eAG) : based on value calculated according to National Glycohemoglobin Standardization Program (NGSP) criteria.

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CHROMATOGRAPHY REPORT

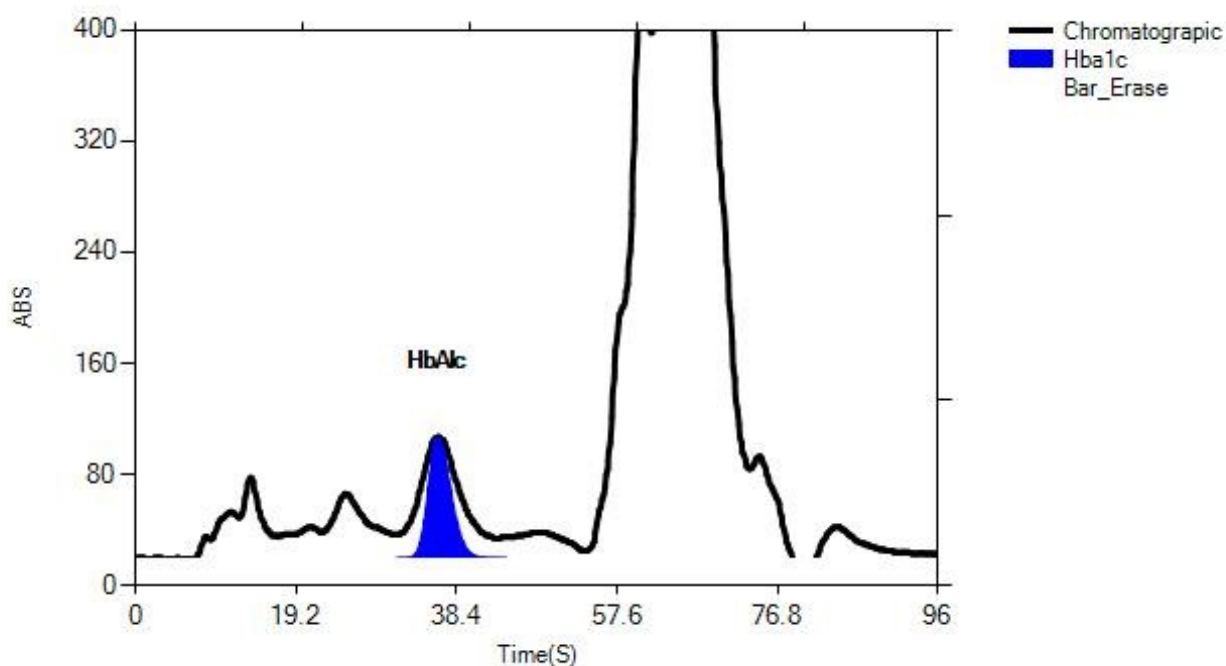
Sample Id : JK3536286

Sample Processed Date: 22/06/2026 / 19:49

Peak Name	Retention Time(s)	Absorbance	Area	Result (Area %)
Hba1a	8.4	16	110	0.05
Hba1b	13.6	59	1529	0.81
HbF	18.3	18	345	0.18
La1c	25.1	47	2362	1.25
HbA1c	36.0	88	7231	5.14
UnKnown	60.5	3.71	6957	3.71
HbA0	65.3	5410	168960	90.11

HbA1c(NGSP)= 5.14

TotalArea= 187493.0



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HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
COMPLETE BLOOD COUNT (CBC), WHOLE BLOOD EDTA			
HAEMOGLOBIN (Hb) Colorimetric method	15.1	g/dL	13.0-17.0
RED BLOOD CELLS- RBC COUNT Electrical impedance	5.22	millions/mm ³	4.5 - 5.5
PACKED CELL VOLUME (PCV) - HEMATOCRIT Automated/Calculated	48.9	%	40.0-50.0
MCV Automated/Calculated	93.8	fL	83-101
MCH Automated/Calculated	29.0	pg	27.0-32.0
MCHC Automated/Calculated	30.9	g/dL	31.5-34.5
RED CELL DISTRIBUTION WIDTH (RDW-CV) Automated/Calculated	14.0	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH (RDW-SD) Automated/Calculated	46.9	fL	39.0- 46.0
MENTZER INDEX Calculated	17.97		
PLATELET COUNT Electrical impedance/Microscopy	130	10 ³ /μL	150-410
PLATELET DISTRIBUTION WIDTH (PDW) Calculated	22.7	fL	9.00-17.00
PCT(PLATELETCRIT) Calculated	0.178	%	0.108-0.282
MEAN PLATELET VOLUME - MPV Calculated	13.6	fL	7.00-12.0
P-LCR Calculated	52.40	%	11.0-45.0
P-LCC Calculated	68.00	10 ⁹ /L	30.0-90.0
TOTAL LEUKOCYTE COUNT (TLC) Electrical impedance	6.28	10 ³ /μL	4.00-10.0
DIFFERENTIAL LEUCOCYTE COUNT			
Neutrophils Flow cytometry/Manual	59.1	%	40 - 80
Lymphocytes Flow cytometry/Manual	29.3	%	20 - 40
Eosinophils Flow cytometry/Manual	2.9	%	1.00-6.00
Monocytes Flow cytometry/Manual	8.6	%	2.00-10.0

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HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
Basophils Flow cytometry/Manual	0.1	%	0.00-1.00
ABSOLUTE NEUTROPHIL COUNT Calculated	3.71	10 ³ /μL	2.00-7.00
ABSOLUTE LYMPHOCYTE COUNT Calculated	1.84	10 ³ /μL	1.00-3.00
ABSOLUTE EOSINOPHIL COUNT Calculated	0.18	10 ³ /μL	0.02-0.50
ABSOLUTE MONOCYTE COUNT Calculated	0.54	10 ³ /μL	0.20-1.00
ABSOLUTE BASOPHIL COUNT Calculated	0.01	10 ³ /μL	0.02-0.10

CLINICAL NOTES

A complete blood count (CBC) is used to evaluate overall health and detect wide range of disorders, including anemia, infection and leukemia. There have been some reports of WBC and platelet counts being lower in venous blood than in capillary blood samples ,although still within these reference ranges.

POSSIBLE CAUSES OF ABNORMAL PARAMETERS

High RBC, Hb, or HCT - dehydration, polycythemia, shock, chronic hypoxia	Low RBC, Hb, or HCT - anemia, thalassemia, and other hemoglobinopathies.
High MCV - macrocytic anemia, liver disease	Low MCV - microcytic anemia.
High WBC -acute stress, infection, malignancies	Low WBC - sepsis, marrow hypoplasia
High platelets - risk of thrombosis	Low platelets - risk of bleeding

Notes

1. Macrocytic Anemia/Dimorphic Anemia can have low platelet count.
2. Microcytic Anemia/Leucocytosis can have Reactive thrombocytosis.

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BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval
CALCIUM-SERUM			
CALCIUM , Serum BAPTA	9.11	mg/dL	8.4 - 10.6

CLINICAL NOTES

A blood calcium test is ordered to screen for, diagnose, and monitor a range of conditions relating to the bones, heart, nerves, kidneys, and teeth. The test may also be ordered if a person has symptoms of a parathyroid disorder, malabsorption, or an overactive thyroid. To help diagnose the underlying problem, additional tests are often done to measure ionized calcium, urine calcium, phosphorus, magnesium, vitamin D, parathyroid hormone (PTH) and PTH-related peptide (PTHrP). PTH and vitamin D are responsible for maintaining calcium concentrations in the blood within a narrow range of values. Measuring urine calcium can help determine whether the kidneys are excreting the proper amount of calcium,

Serum calcium is decreased (hypocalcemia) in following conditions-

- Hypoparathyroidism, Pseudohypoparathyroidism
- Vitamin D deficiency (either from intake deficiency or decreased conversion/activation) or resistance (osteomalacia and rickets)
- Chronic renal diseases (eg, renal acidosis, Fanconi syndrome), Chronic liver disease and biliary obstructive diseases
- Magnesium deficiency (PTH glandular release is magnesium-dependent), Hyperphosphatemia, Hypoalbuminemia
- Overexpression of fibroblast growth factor 23 (oncogenic osteomalacia)
- Severe calcium dietary deficiency, Hungry bone syndrome, Severe pancreatitis (calcium saponification), Massive transfusion

Serum calcium is Increased (hypercalcemia) in following conditions-

- Hyperparathyroidism (primary, such MEN type 1, hyperplasia, adenoma, or carcinoma; or secondary, from chronic kidney injury and hyperphosphatemia)
- Malignancies (humoral hypercalcemia of malignancy) that secrete PTH-related protein, especially squamous cell carcinoma of lung and renal cell carcinoma
- Vitamin D excess, Vitamin A intoxication, Milk-alkali syndrome
- Multiple myeloma, owing to bone lesions, Paget disease of bone with prolonged immobilization, Sarcoidosis, Other granulomatous disorders
- Familial hypocalciuric hypercalcemia, Addison disease
- Thyrotoxicosis, Hypothyroidism, owing to prolongation of vitamin D action as its metabolism is slowed down
- Drug exposure: Some drugs that can increase serum calcium are as follows antacids (some), calcium salts, long-term thiazide therapy, lithium

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BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval
BLOOD GLUCOSE FASTING			
BLOOD GLUCOSE FASTING,Plasma Sod.F Hexokinase	76.90	mg/dL	59-100

CLINICAL NOTES

Elevated glucose levels (hyperglycemia) are most often encountered clinically in the setting of diabetes mellitus, but they may also occur with pancreatic neoplasms, hyperthyroidism, and adrenocortical dysfunction. Decreased glucose levels (hypoglycemia) may result from endogenous or exogenous insulin excess, prolonged starvation, or liver disease.

Fasting Glucose	2 HOURS PP Glucose	Diagnosis
<100	<140	Normal
100 to 125	140 to 199	Pre Diabetes
>126	>200	Diabetes

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

Note: Blood glucose level is maintained by a very complex integrated mechanism involving a critical interplay of the release of hormones and action of enzymes on key metabolic pathways. If postprandial glucose is lower than fasting glucose, it is termed as postprandial reactive hypoglycemia (PRH). The possible cause of PRH are high insulin sensitivity, exaggerated response of insulin and glucagon-like peptide 1, defects in counter-regulation, very lean individuals, anxious individuals, after massive weight reduction, women with lower body overweight physical activity prior test, hypoglycemic medication, deliberately eating less or eat a non-carbohydrate meal before testing.

Ref : American Diabetes association standards of medical care.

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BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval
<u>LIVER FUNCTION TEST (LFT) - EXTENDED</u>			
BILIRUBIN TOTAL, Serum Diazonium Ion	0.44	mg/dl	0.010 - 1.20
BILIRUBIN (Direct), Serum Diazotization Method	0.12	mg/dL	0.00-0.20
INDIRECT BILIRUBIN, Serum Calculated	0.32	mg/dL	0.80
SGPT (ALT), SERUM UV without P5P	38.60	U/L	0-35
SGOT (AST) ,SERUM UV without P5P	27.70	U/L	0-40
ALKALINE PHOSPHATASE ,Serum IFCC	110.0	U/L	53-128
GAMMA GLUTAMYL TRANSFERASE (GGT), Serum IFCC	23.00	U/L	12.0-58.0
TOTAL PROTEIN , Serum Biuret	7.35	g/dL	6.00-8.30
Albumin, Serum BCG	4.53	g/dL	3.2-5.20
GLOBULIN, SERUM Calculated	2.82	g/dL	2.30-4.50
A/G Ratio ,Serum Calculated	1.61		1.0 - 2.3
SGOT/SGPT RATIO	0.72		

Comment:

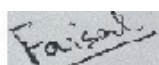
Kindly correlate clinically.

COMMENT

These are group of tests that can be used to detect the presence of liver disease, distinguish among different types of liver disorders, gauge the extent of known liver damage, and monitor the response to treatment. Most liver diseases cause only mild symptoms initially, but these diseases must be detected early. Some tests are associated with functionality (e.g., albumin), some with cellular integrity (e.g., transaminase), and some with conditions linked to the biliary tract (gamma-glutamyl transferase and alkaline phosphatase). Conditions with elevated levels of ALT and AST include hepatitis A,B ,C ,paracetamol toxicity etc. Several biochemical tests are useful in the evaluation and management of patients with hepatic dysfunction. Some or all of these measurements are also carried out (usually about twice a year for routine cases) on those individuals taking certain medications, such as anticonvulsants, to ensure that the medications are not adversely impacting the person's liver.

Reference ranges are from Teitz fundamental of clinical chemistry 8th ed (2018)

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BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval
KIDNEY FUNCTION TEST (KFT)-BASIC			
UREA - SERUM Urease UV	28.0	mg/dL	19.0 - 44.0
CREATININE-SERUM Jaffe Kinetic	0.78	mg/dL	0.60-1.30
URIC ACID - SERUM URICASE-POD	5.70	mg/dL	3.50 - 7.20
SODIUM (SERUM) ISE	142.3	mmol/L	135 - 150
POTASSIUM-SERUM ISE	4.25	mmol/L	3.5 - 5.5
CHLORIDE ,Serum ISE	103.20	mmol/L	94 - 110
BLOOD UREA NITROGEN (BUN) Calculated	13.08	mg/dL	8.00-23.0
BUN/CREATININE RATIO Calculated	16.77	Ratio	10-20:1 Normal
UREA / CREATININE RATIO Calculated	35.90	Ratio	40-100:1 Normal

INTERPRETATION

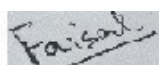
Kidney function tests are group of tests that can be used to evaluate how well the kidneys are functioning. Creatinine is a waste product produced by muscles from the breakdown of a compound called creatine. In blood, it is a marker of GFR, in urine, it can remove the need for 24-hour collections for many analytes or be used as a quality assurance tool to assess the accuracy of a 24-hour collection. It is removed from the body by the kidneys, which filter almost all of it from the blood and release it into the urine. This test measures the amount of creatinine in the blood and/or urine. Creatine is part of the cycle that produces energy needed to contract muscles. Both creatine and creatinine are produced by the body at a relatively constant rate. Since almost all creatinine is filtered from the blood by the kidneys and released into the urine, blood levels are usually a good indicator of how well the kidneys are working.

REMARK-The amount of creatinine you produce depends on your body size and your muscle mass. For this reason, creatinine levels are usually slightly higher in men than in women and children. Certain drugs are nephrotoxic hence KFT is done before and after initiation of treatment with these drugs.

Higher creatinine than normal level may be due to: • Blockage in the urinary tract • Kidney problems, such as kidney damage or failure, infection, or reduced blood flow • Loss of body fluid (dehydration) • Muscle problems, such as breakdown of muscle fibers • Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)

Lower than normal creatinine level may be due to: • Myasthenia Gravis • Muscular dystrophy. Low serum creatinine values are rare; they almost always reflect low muscle mass.

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BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval
LIPID PROFILE BASIC.			
CHOLESTEROL TOTAL - Serum Cholesterol Oxidase, Esterase, Peroxidase	168.00	mg/dL	<200 Desirable 200-239 Borderline high risk >240 High risk
TRIGLYCERIDES - SERUM GPO-POD	218.70	mg/dL	<150
CHOLESTEROL - HDL (DIRECT) IMMUNOINHIBITION	40.90	mg/dL	>40 Recommended Range
NON-HDL CHOLESTEROL	127.10	mg/dL	<130
CHOLESTEROL-LDL (DIRECT) Calculated	83.36	mg/dL	<130 Recommended Range
VLDL ,SERUM Calculated	43.74	mg/dL	0.00 - 45.0
CHOL / HDL RATIO	4.11		
LDL/HDL Ratio Calculated	2.04	Ratio	1.0-3.5
HDL/LDL CHOLESTEROL RATIO Calculated	0.49	Ratio	<3.50

Comment: Kindly correlate clinically.

REFERENCE RANGES AS PER NCEP ATP III GUIDELINES

TOTAL CHOLESTEROL	mg/dl	HDL	mg/dl	LDL	mg/dl	TRIGLYCERIDES	mg/dl
Desirable	<200	Low	<40	Optimal	<100	Normal	<150
Borderline High	200-239	High	>60	Near Optimal	100-129	Borderline High	150-199
				BorderlineHigh	130-159		
High	>240	-	-	High	160-189	High	200-499
				Very High	>190	Very High	>500

TI!! 10-12 hours fasting is mandatory for lipid parameters.If not, values might fluctuate

CLINICAL NOTES-Lipid profile is initial screening tool for abnormalities in lipids. The results of this test can identify certain genetic diseases & can determine approximate risks for cardiovascular disease, certain forms of pancreatitis. Hypertriglyceridemia is indicative of insulin resistance when present with low HDL & elevated LDL, while elevated TG is risk factor for coronary artery disease, especially when low HDL is present. TG of 500mg/dL or more can be concerning for development of pancreatitis. ***The calculated value for LDL-C is typically reported as part of the lipid profile as per friedewald equation. When triglycerides are high(>350mg/dl), the equation is no longer valid. In this situation, the only way to accurately determine LDL-C is to measure it directly.**

Remark-Measurements in the same patient can show physiological & analytical variations. 3 serial samples 1 week apart are recommended for Total Cholesterol, TG, HDL & LDL Cholesterol. As per NCEP guidelines, all adults above the age of 20 years should be screened for lipid status. Selective screening of children above the age of 2 years with a family history of premature cardiovascular disease or those with at least one parent with high total cholesterol is recommended. NCEP Identifies elevated Triglycerides as an independent risk factor for Coronary Heart Disease (CHD). Ref: Friedewald WT, Levy RI, Fredrickson DS. Estimation of the concentration of low-density lipoprotein cholesterol in plasma, without use of the preparative ultracentrifuge. Clin Chem. 1972; 18:499-502. PubMed ID: 4337382)

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C. NO. 305023071705Q

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Patient Name: **Mr. ZAHIQ ABBAS / TUJER** PateintID : **LDPL/01076921**
Age/Gender : 23 Y/Male Visit No : **SRI2606220995**
Refer Client : LDPLJK2347 ZAINGAIR BLOOD COLLECTION C Collected On : 22/Jun/2026 05:08 PM
Referred By : N/A Received On : 22/Jun/2026 07:35 PM
Doctor Name: Reported Date 22/Jun/2026 10:00 PM
SampleType : Serum - JK3536287, Sod.Fluoride - F - JK3536285, Urine - , Whole Blood EDTA - JK3536286



BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval
IRON PROFILE BASIC			
IRON -Serum Ferrozine-no Deproteinization	139.85	ug/dL	59.0-158.0
UIBC-SERUM NiTRO-PSAP	232.40	ug/dL	110 - 370
TOTAL IRON BINDING CAPACITY Calculated	372.25	ug/dL	240-450
TRANSFERRIN SATURATION Calculated	37.57	%	20.0-50.0

CLINICAL NOTES

The serum iron test is used to measure the amount of iron that is in transit in the body – the iron that is bound to transferrin in the blood. Along with other tests, it is used to help detect and diagnose iron deficiency or iron overload. Testing may also be used to help differentiate various causes of anemia. The amount of iron present in the blood will vary throughout the day and from day to day. For this reason, serum iron is almost always measured with other iron tests, including ferritin, transferrin, and calculated total iron-binding capacity (TIBC) and transferrin saturation. Serum ferritin appears to be in equilibrium with tissue ferritin and is a good indicator of storage iron in normal subjects and in most disorders. In patients with some hepatocellular diseases, malignancies and inflammatory diseases, serum ferritin is a disproportionately high estimate of storage iron because serum ferritin is an acute phase reactant. In such disorders iron deficiency anemia may exist with a normal serum ferritin conc. In the presence of inflammation, persons with low serum ferritin are likely to respond to iron therapy.

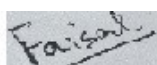
Increased Levels

-Iron overload – Hemochromatosis, Thalassemia & Sideroblastic anemia
-Malignant conditions - Acute myeloblastic & Lymphoblastic leukemia, Hodgkin's disease & Breast carcinoma
-Inflammatory diseases - Pulmonary infections, Osteomyelitis, Chronic UTI,
-Rheumatoid arthritis, SLE, burns, Acute & Chronic hepatocellular disease

Decreased Levels

-Iron deficiency anemia

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SampleType : Serum - JK3536287,Sod.Fluoride - F - JK3536285,Urine - ,Whole Blood EDTA - JK3536286

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Visit No : SRI2606220995

Collected On : 22/Jun/2026 05:08 PM

Received On : 22/Jun/2026 07:35 PM

Reported Date 22/Jun/2026 09:52 PM



IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
VITAMIN B12 : CYANOCOBALAMIN			
VITAMIN B12 : CYANOCOBALAMIN,Serum ECLIA	169.00	pg/mL	211 - 911

Comment: Kindly correlate clinically.

CLINICAL NOTES

Vitamin B12 performs many important functions in the body, but the most significant function is to act as coenzyme for reducing ribonucleotides to deoxyribonucleotides, a step in the formation of genes. Inadequate dietary intake is not the commonest cause for cobalamine deficiency. The most common cause is malabsorption either due to atrophy of gastric mucosa or diseases of terminal ileum. Cobalamine deficiency leads to Megaloblastic anemia and demyelination of large nerve fibres of spinal cord. Sources of Vitamin B12 are liver, shellfish, fish, meat, eggs, milk, cheese & yogurt.

Decreased Levels

- Dietary deficiency: Vegetarians
- Lack of Intrinsic factor: Total or partial gastrectomy, Atrophic gastritis, Intrinsic factor antibodies
- Malabsorption: Regional ileitis, resected bowel, Tropical Sprue, Celiac disease, pancreatic insufficiency, bacterial overgrowth & achlorhydria
- Loss of ingested vitamin B12: fish tapeworm
- Congenital disorders: Orotic aciduria & transcobalamine deficiency
- Increased demand: Pregnancy specially last trimester

Increased Levels

- Chronic renal failure, Congestive heart failure, Acute & Chronic Myeloid Leukemia, Polycythemia vera, Carcinomas with liver metastasis, Liver disease, Drug induced cholestasis & Protein malnutrition

Note: To differentiate vitamin B12 & folate deficiency, measurement of Methyl malonic acid in urine & serum Homocysteine level is suggested

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Doctor Name:

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IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
<u>VITAMIN D 25-HYDROXY CHOLECALCIFEROL</u>			
VITAMIN D(25 OH) ,SERUM ECLIA	10.20	ng/mL	Deficiency <20 Insufficiency 20-30 Sufficiency 30-100 Toxicity >100

Comment: Kindly correlate clinically.

CLINICAL NOTES-

Vitamin D is essential for strong bones, because it helps the body use calcium from the diet. Traditionally, vitamin D deficiency has been associated with rickets, a disease in which the bone tissue doesn't properly mineralize, leading to soft bones and skeletal deformities. But increasingly, research is revealing the importance of vitamin D in protecting against a host of health problems.

Symptoms and Health risks of vitamin D deficiency

Symptoms of bone pain and muscle weakness can mean you have a vitamin D deficiency. However, for many people, the symptoms are subtle. Yet, even without symptoms, too little vitamin D can pose health risks. Low blood levels of the vitamin have been associated with Increased risk of cardiovascular disease, Cognitive impairment in older adults, Severe asthma in children & Cancer.

Research suggests that vitamin D could play a role in the prevention and treatment of a number of different conditions, including type 1 and type 2 diabetes, hypertension, glucose intolerance, and multiple sclerosis.

Causes of vitamin D deficiency

- People following a strict vegan diet.
- Exposure to sunlight is limited,
- Vitamin D malabsorption- like Crohn's disease, cystic fibrosis, and celiac disease,
- Dietary deficiency
- Severe Hepatocellular disease
- Drugs like Anticonvulsants
- Nephrotic syndrome

Increased levels

- Vitamin D intoxication

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IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
THYROID PROFILE : T3, T4 & TSH(TFT)			
TRIODOOTHYRONINE TOTAL (T3),Serum ECLIA	0.80	ng/mL	0.70-2.04
THYROXINE TOTAL (T4),Serum ECLIA	8.59	ug/dl	4.6-10.5
THYROID STIMULATING HORMONE (TSH),Serum ECLIA	4.551	μIU/ml	0.35-5.50

NOTE-TSH levels are subject to circadian variation, reaching peak levels between 2-4 AM and min between 6-10 PM. The variation is the order of 50% hence time of the day has influence on the measures serum TSH concentration. Dose and time of drug intake also influence the test result.

Transient increase in TSH levels or abnormal TSH levels can be seen in some non thyroidal conditions, simultaneous measurement of TSH with free T4 is useful in evaluating differential diagnosis.

INTERPRETATION-Ultra Sensitive 4th generation assay

1.Primary hyperthyroidism is accompanied by ↑serum T3 & T4 values along with ↓ TSH level.

2.Low TSH, high FT4 and TSH receptor antibody (TRAb) +ve seen in patients with Graves disease

3.Low TSH, high FT4 and TSH receptor antibody (TRAb) -ve seen in patients with Toxic adenoma/Toxic Multinodular goiter

4.High TSH, Low FT4 and Thyroid microsomal antibody increased seen in patients with Hashimoto's thyroiditis

5.High TSH, Low FT4 and Thyroid microsomal antibody normal seen in patients with Iodine deficiency/Congenital T4 synthesis deficiency

6.Low TSH, Low FT4 and TRH stimulation test -Delayed response seen in patients with Tertiary hypothyroidism

7.Primary hypothyroidism is accompanied by ↓ serum T3 and T4 values & ↑serum TSH levels

8.Normal T4 levels accompanied by ↑ T3 levels and low TSH are seen in patients with T3 Thyrotoxicosis

9.Normal or ↓ T3 & ↑T4 levels indicate T4 Thyrotoxicosis (problem is conversion of T4 to T3)

10.Normal T3 & T4 along with ↓ TSH indicate mild / Subclinical Hyperthyroidism .

11.Normal T3 & ↓ T4 along with ↑ TSH is seen in Hypothyroidism .

12.Normal T3 & T4 levels with ↑ TSH indicate Mild / Subclinical Hypothyroidism .

13.Slightly ↑ T3 levels may be found in pregnancy and in estrogen therapy while ↓ levels may be encountered in severe illness , malnutrition , renal failure and during therapy with drugs like propranolol.

14.Although ↑ TSH levels are nearly always indicative of Primary Hypothyroidism , rarely they can result from TSH secreting pituitary tumours.

DURING PREGNANCY - REFERENCE RANGE for TSH in uIU/mL (As per American Thyroid Association)

1st Trimester : 0.10-2.50 uIU/mL

2nd Trimester : 0.20-3.00 uIU/mL

3rd Trimester : 0.30-3.00 uIU/mL

The production, circulation, and disintegration of thyroid hormones are altered throughout the stages of pregnancy.

REMARK-Assay results should be interpreted in context to the clinical condition and associated results of other investigations. Previous treatment with corticosteroid therapy may result in lower TSH levels while thyroid hormone levels are normal. Results are invalidated if the client has undergone a radionuclide scan within 7-14 days before the test. Abnormal thyroid test findings often found in critically ill patients should be repeated after the critical nature of the condition is resolved. TSH is an important marker for the diagnosis of thyroid dysfunction. Recent studies have shown that the TSH distribution progressively shifts to a higher concentration with age ,and it is debatable whether this is due to a real change with age or an increasing proportion of unrecognized thyroid disease in the elderly.

End of Report

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